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Development of the Mental Health Triage Tool

Tools to aid the triage of people presenting to EDs with psychological distress or acute behavioural disturbance have been developed and refined in Australia from the 1990s. These include guidelines in NSW and a triage rating scale in Tasmania.[14,15] A mental health triage tool was developed by South Eastern Sydney Area Health Services and further refined by Broadbent et al.[16,17]

Risk factors for suicide

Consider risk factors for suicide when deciding on the triage category, where the person should wait and whether to flag the person's risk to senior staff. See Common risk factors for suicide (below).

Key point

You should ask the person whether they are having suicidal thoughts – asking this does not increase the risk of suicide.[5] Sensitively framed, but direct, questions are best.

Common risk factors for suicide

All ages

- Mood disorder
- Suicidal ideation
- Relationship problems
- Alcohol and drug problems, intoxication
- History of self-harm
- Anxiety and stress disorders
- Death of a family member
- Legal or employment problems
- Male gender
- Access to means to carry out suicide
- Living in a regional or remote area
- Stressors for First Nations people, such as racism and trauma
- Limitation of activities due to disability
- Pain
- Hopelessness and helplessness.

Adolescents

- Hostility, aggression and impulsivity
- Stressors related to sexuality or gender identity
- Recent commencement of antidepressant therapy
- Close friends who have died by suicide.

Older people

- Social isolation
- Concerns about being a burden to other people
- Tension with caregivers
- Recent change in accommodation.

Sources: *Causes of Death, Australia 2021*[11]; *Demoralization in Suicide: A systematic review*[12]; *Working with the Suicidal Person – Clinical Practice Guidelines for Emergency Departments and Mental Health Services*[13]

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Table 5.1: Useful assessment questions for patients experiencing psychological distress or behavioural disturbance

Question	Comment
Understanding the presenting problem	
Things seem to be difficult for you at the moment – can you tell me more about it? I sense that you are feeling worried – do you feel safe? I can see that you are feeling angry/anxious – can you tell me why?	These are open-ended questions to encourage people experiencing distress to talk about their difficulties[5] If these strategies don't work, the person's silence may indicate serious illness – they should never feel forced to speak
Has anything happened today or recently that made you need help from us?	Stressful situations can increase the risk of suicide (e.g. relationship problems, employment-related or legal issues, bereavement)[11]
I have to ask some sensitive questions now: Do you have any thoughts of suicide now? If yes: do you have any plans to do this? Do you have access to what you would need to do this? Have you tried to take your life today?	Yes to any of these questions increases the risk of suicide[11]
Have you tried, or wanted, to hurt yourself? If yes: is this to release emotions?	Self-harm to release emotions has a different intent to suicide
Do you have any thoughts of harming someone else? If yes: do you have a specific person in mind?	Threat of harm to others suggests a triage category of 2
Do you hear voices when there is no one else around or have other unusual experiences you can't explain? If yes: are these voices telling you to do things? If yes: do you feel like you have to do what the voices tell you?	Command hallucinations that the person is unable to resist suggest a triage category of 1. The person may not be able to tell you about these experiences – do not press them
Did someone bring you here?	People brought in by police, paramedics or mental health workers may not have been able to identify their own health needs People presenting alone may be at greater risk of leaving[2]

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Table 5.1: Useful assessment questions for patients experiencing psychological distress or behavioural disturbance (continued)

Question	Comment
Identify factors that may impact on the presenting problem	
Do you take any prescribed medication? If yes: have you been taking it? How much have you been taking? Do you have any medication with you?	If the person has medication with them, ask if you can take it from them
Do you take any non-prescribed medications or other drugs, or drink alcohol? If yes: have you had any today and if so how much?	Intoxication is a risk factor for suicide and reduces capacity to make an informed decision to not wait Alcohol and substance-related disorders increase the risk of suicide[11]
Have you hurt yourself in the past? If yes: was this to release emotions or did you want everything to end?	This can help understand a person's suicide risk
Have you experienced mental illness in the past? If yes: is this presentation similar or different? Have you attempted suicide in the past? Do you have other medical conditions?	This context can help you understand the person's needs and suicide risk
Do you feel safe to stay? Do you feel safe to tell me if anything changes?	This can help determine an appropriate location post-triage

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Table 5.2: Mental Health Triage Tool – summary

	Category 1 Immediate	Category 2 Emergency	Category 3 Urgent	Category 4 Semi-urgent	Category 5 Non-urgent
Description	Definite danger to life (self or others)	▪ Probable risk of danger to self or others, and/or ▪ Client is physically restrained in ED, and/or ▪ Severe behavioural disturbance	Possible danger to self or others: ▪ Moderate behaviour disturbance ▪ Severe distress	Moderate distress	No danger to self or others
Typical presentation features	**Observed** ▪ Violent behaviour ▪ Possession of a weapon ▪ Major self-harm in the ED ▪ Extreme agitation or restlessness ▪ Bizarre/ disoriented behaviour **Reported** ▪ Verbal commands to do harm to self or others, that the person is unable to resist (command hallucinations) ▪ Recent violent behaviour	**Observed** ▪ Extreme agitation/ restlessness ▪ Physically/verbally aggressive ▪ Confused/unable to cooperate ▪ Hallucinations/ delusions/paranoia ▪ Requires restraint/ containment ▪ High risk of absconding and not waiting for treatment ▪ Unable to wait safely **Reported** ▪ Attempt at self-harm/threat of self-harm ▪ Threat of harm to others	**Observed** ▪ Agitation/restlessness ▪ Intrusive behaviour ▪ Confusion ▪ Abivalence about treatment ▪ Not likely to wait for treatment ▪ Hallucinations/delusions/ paranoia ▪ Thought disorder ▪ Bizarre/agitated behaviour ▪ Severe symptoms of depression ▪ Withdrawn/ uncommunicative and/or anxiety ▪ Elevated or irritable mood **Reported** ▪ Suicidal ideation ▪ Situational crisis	**Observed** ▪ No agitation/ restlessness ▪ Irritable without aggression ▪ Cooperative ▪ Gives coherent history **Reported** ▪ Pre-existing mental health disorder ▪ Symptoms of anxiety or depression without suicidal ideation ▪ Willing to wait	**Observed** ▪ Cooperative ▪ Communicative and able to engage in developing management plan ▪ Able to discuss concerns **Reported** ▪ Known patient with chronic psychotic symptoms ▪ Pre-existing non-acute mental health disorder ▪ Known patient with chronic unexplained somatic symptoms ▪ Request for medication ▪ Minor adverse effect of medication ▪ Financial, social, accommodation, or relationship problems

Source: Adapted from the Mental Health Triage Tool